

Prior Authorization Request

Health Plan

Humana Medicare Advantage -- Gold Choice PPO.

Member Information

Member: Otilie M. Ranganathan DOB: 1957-12-19 (age 68) Member ID: MBR-73015

Sex: F

Requesting Provider

Practice: Kestrel Point Cardiology

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-11-18

Level of care requested: INPATIENT

Made-up data for a software prototype. Not a real patient.

Heart failure consultation

KESTREL POINT CARDIOLOGY | Advanced Heart Failure

Seen: 21-Sep-2026 Patient: Ranganathan, Otilie M. DOB 19-Dec-1957

History: Mrs. Ranganathan developed exertional breathlessness in summer 2025. Admitted Aug 2025, found to have a dilated LV with EF in the 20s. Coronary angiogram 14-Aug-2025 showed smooth, normal coronaries. Started on full guideline therapy 18-Aug-2025 and titrated over the autumn.

Today she can do her shopping but must sit down after climbing one flight. No breathlessness at rest. No blackouts.

Echo today: LVEF 27%.

Impression:

- Dilated cardiomyopathy, non-ischaemic. NYHA functional class II.
- CKD stage 3b (creatinine 1.7).
- Type 2 diabetes on basal insulin.

Recommendation: primary prevention ICD. Referred for a shared decision-making visit and to Dr. Varga-Holm for implant.

Dr. Eleni Papadakis-Moore

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Echocardiogram report

KESTREL POINT CARDIOLOGY - ECHO LAB

Date: 21-Sep-2026

Pt: RANGANATHAN OTTILIE M

LVEF (biplane): 27 %

LVEDD 6.4 cm. Global hypokinesis. Moderate functional MR.

RV size and function normal. PASP 36 mmHg.

Conclusion: severe LV systolic impairment, essentially unchanged from 03/2026 (LVEF 26%).

Sonographer: K. Odhiambo Reporting physician: R. Lindvall, MD

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Coronary angiogram report (2025)

KESTREL POINT REGIONAL HOSPITAL - CATH LAB

Procedure date: 14-Aug-2025

Ranganathan, Otilie

Indication: new LV dysfunction

Left main, LAD, circumflex and RCA all angiographically normal. Right dominant.

Conclusion: normal coronary arteries. Non-ischaemic cause of LV dysfunction. No intervention.

Operator: S. Achterhof, MD

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Current medications

MEDICATION RECORD - Ranganathan, Otilie M. - printed 21-Sep-2026

Sacubitril/valsartan 97/103 mg twice daily	commenced 18-Aug-2025
Bisoprolol 7.5 mg daily	commenced 18-Aug-2025
Eplerenone 25 mg daily	commenced 18-Aug-2025
Dapagliflozin 10 mg daily	commenced 18-Aug-2025
Insulin degludec 24 units nightly	commenced 2021
Furosemide 40 mg mornings	commenced Aug-2025
Ferrous sulfate 325 mg daily	commenced 2024

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Shared decision-making visit

KESTREL POINT CARDIOLOGY - DEVICE DECISION VISIT

Date: 29-Sep-2026

Patient: Otilie Ranganathan, attended with her son

Clinician: Harriet Ojukwu, ANP

We used a published, evidence-based ICD decision aid (paper version) and went through it page by page.

Topics: how often an ICD saves a life in someone like her, shocks (right and wrong), infection, lead faults, generator changes, switching it off near end of life, and the choice of no device.

She said in her own words that the device protects against sudden death but will not help her breathing.

Her choice: ICD.

H. Ojukwu ANP

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Admission plan for device implant

KESTREL POINT CARDIOLOGY - ELECTROPHYSIOLOGY ADMISSION PLAN

Prepared: 02-Oct-2026

Patient: Ranganathan, Otilie M. MBR-73015

Planned procedure: single-chamber ICD, CPT 33249. Admission 18-Nov-2026 (inpatient).

Anticipated stay: 2 midnights.

Reasons: CKD 3b needing renal function check after contrast venogram; insulin-treated diabetes needing glucose management around fasting; iron-deficiency anaemia (Hb 10.6).

History of MI, coronary stent or bypass: none.

Contraindications reviewed: no dementia, no illness expected to limit life to under a year, no atrial fibrillation, clinically stable. None present.

Dr. Ilona Varga-Holm

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